POSTette: Discharge Summary: Documentation and Clinical Considerations

Reference: Medicare Benefit Manuals Chapters 8 & 15; Therapy Policy 250

Discharge Summary
Refer to Rehabilitation P&P 250 Discharge Summary for clinical practice guidelines.

A discharge summary is required for every episode of treatment. Emphasis needs to be on describing the medical necessity and justification of services provided especially any services provided beyond those usually expected for any given condition.

Discharge summaries are to be written by the clinician (with input and support from assistants when allowed by state, federal practice) and is the final opportunity to summarize the clinical picture for that patient (think of a reviewer reading the documentation) including:

- Diagnosis
- Functional deficits
- Progress (or lack of)
- Medical necessity
- Justification
- Patient response
- Patient's current level of function

A discharge summary covers the last reporting period and includes information about the entire course of treatment. Think of the discharge summary as the last written document to support the provided services.

NOTE: A good standard of practice (required by some states) is to include a TEN on the last known date of service addressing the scheduled discharge.

Provide a description of any complicating factors that impacted progress.
For example:
Mr. Jones suffered an acute UTI during the first week of therapy and his level of progress regressed. However, during the last two weeks of skilled intervention he made additional gains and was able to meet the goal for independent ambulation.

It is imperative to include objective comparisons of functional tasks from baseline measures during initial evaluation to the discharge status. Along with including information on the goals achieved, provide an explanation for any goals not met during the course of treatment.

Take the time to emphasize the skilled services and the treatment methods provided.
For example:
Initially therapy focused on progressive-resistive exercises for lower extremities to stabilize his stance and balance. As his strength and balance improved, therapy expanded to address his foot drag and focus on proper placement of his walker and sequence of steps.

Be sure to document any specific recommendations or trainings provided to the caregiver (patient, family, or nursing staff) to ensure continuity of care. Training of caregivers is essential for a smooth transition off of therapy services, especially if the patient is returning home or to a lesser level of care. Also include a summary of any equipment provided or still needed.

Conclude with a brief statement of how skilled intervention has improved the patient's function and/or quality of life.

For example:
As a result of skilled therapy intervention, Mr. Jones can now ambulate safely and independently to be able to join other patients in activities outside of the facility.

Think of the discharge note as an opportunity to describe the outcomes of skilled intervention from start to finish. It is the final piece of an organized documentation system that is necessary in the event the record is reviewed.

Documentation serves as a permanent record of the interventions that were provided and need to stand the test of time.

In the event of a record review, the information documented is the only evidence of what the deficits were, what interventions were provided, the patient's response to those interventions, and the functional status at discharge.

If therapists do not capture this information at the time services occurred, then they run the risk of having the services denied by a reviewer. The most common reasons for denials outside of "technical issues" are: services are not skilled, no statement of functional goals, amount/frequency/duration was not reasonable, or documentation does not support the minutes logged.

Following the basic documentation guidelines will put therapists on the right path to meeting the requirements of third party payers.

Understanding what is required it takes the guess work out and allows more time for direct patient care.

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Clinical Considerations when Discharge Planning

Discharge planning starts at admission with the IDT focused on the quality of care transitions and discharge planning.

- What was the PLOF and will the patient return to that level? What will the patient need to be able to do?
- Who are the caregivers? Caregiver education is an integral part of any skilled treatment plan to ensure carryover of progress and to help minimize the effects of potential decline.
- What are the barriers?
- What services / equipment will be required at discharge?

During the course of treatment review the data collected and any relevant standard of practice/evidence based measures that tie back to goals and outcomes. The data should be captured into the documentation in a way that informs the IDT and caregivers of progress and problem areas to address.

Utilize clinical pathways that highlight important follow-up and self-management instructions, and any pending diagnostics/treatments. Consider the following prior to discharge:

- Patient education
- Levels of performance at discharge compared to admission
- Skilled services provided
- Recommendations for any services/equipment needed

REMEMBER:
Skilled intervention can go beyond the walls of the facility for patients to safely return to community living. Skilled intervention needs to prepare patients for all potential barriers and challenges that may be presented within their discharge environment. The interdisciplinary team can work together to develop functional goals specific to a patient's prior level of independence, current diagnosis, and family support. The ultimate goal for each patient, regardless of level or type of injury, is to regain as much independence as possible prior to discharge.

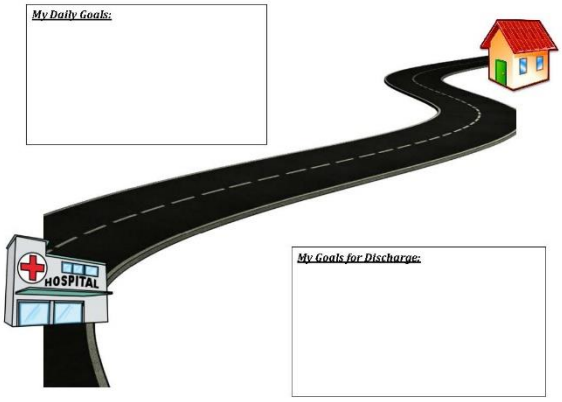
Utilize Discharge Checklists / Home Environment Check List Tools to ensure a patient's level of performance matches what is required in their discharge setting.

The Portal has a variety of tools available:

- Discharge Check list
- Home Environment checklist – for therapists
- Home Environment check list – for families

Another helpful tool is setting goals with the patient by using a discharge setting roadmap. This allows the patient to express the goals that are important to them to get to the next level of care. These can be reviewed weekly along with the therapy goals for ongoing consistent communication.

Discharge setting roadmap example:



Other Resources:

- Home Environment Checklist POSTette
- Community Re-integration POSTette
- Discharge to LTC POSTette

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